

Open Letter to the Honourable Lena Diab

Minister of Immigration, Refugees and Citizenship Canada

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The Honourable Lena Diab, P.C., M.P.
Minister of Immigration, Refugees and Citizenship Canada
Halifax West, Nova Scotia

Dear Minister Diab,

This letter is addressed to you on behalf of concerned Canadians who have followed your conduct as Minister of Immigration, Refugees and Citizenship Canada with growing alarm. We write not as your political opponents, not as partisans, and not with any interest in your personal or political humiliation. We write because the evidence accumulated through parliamentary records, Parliamentary Budget Officer analysis, and your own department's program documentation compels us to do so.

We write because providing false or materially misleading information to a parliamentary committee is not a bureaucratic error. It is a breach of the foundational compact between a minister of the Crown and the Parliament of Canada — and, by extension, the Canadians whom that Parliament represents.

We write because six million Canadians cannot find a family doctor, and when a member of Parliament asked you, at a committee specifically dedicated to the health of Canadians, whether immigration levels were being calibrated against healthcare system capacity, you laughed.

We write because this is not acceptable. Not from any minister. Not in a country facing the healthcare crisis Canada faces today. And we believe that Canadians who read this

letter deserve to know precisely what was said, what the evidence shows, and what we are asking you to do.

I. What Happened: The Record as Established by Evidence

1. The Laughing Dismissal — October 29, 2024

On October 29, 2024, you appeared before the House of Commons Standing Committee on Health. Conservative MP Michael Cooper asked you directly about the relationship between immigration levels and Canada's healthcare system capacity. Your response, as recorded in the committee transcript, included laughter and the statement that this was "not a fair question for immigration."

Minister Diab, we want to be precise about why this matters.

At the time of that hearing, according to Canadian Medical Association data, approximately six million Canadians — citizens, permanent residents, people who have built their lives and paid their taxes in this country — had no family doctor. Emergency departments in rural and Northern communities were closing. Hospitals across the country were operating over capacity. Healthcare professionals were sounding alarms about a system under strain it had not been designed to bear.

A minister of the Crown, appearing before the committee responsible for examining the health of Canadians, laughed at the question of whether those realities were being considered in immigration planning.

This is not a communications failure. It is not a gaffe. It is a window into a ministerial posture that treats the lived suffering of millions of Canadians as an inconvenient intrusion on a preferred political narrative. Canadians experiencing the consequences of a broken healthcare system were watching that committee. They saw your response. We owe it to them to say plainly: that response was beneath the standard every Canadian has a right to expect from a sitting Cabinet Minister.

2. The False Testimony: Rejected Asylum Claimants and IFHP Eligibility

At the same October 29, 2024 Health Committee appearance, you told the committee that rejected asylum claimants are not eligible for premium Interim Federal Health Program (IFHP) benefits.

That statement was false. The evidence is not ambiguous, and it does not come from your political opponents. It comes from three independent and authoritative sources:

Source One — The Parliamentary Budget Officer: The Parliamentary Budget Officer (PBO) — an officer of Parliament whose sole mandate is to provide independent, non-partisan financial analysis to Parliament — published analysis of IFHP costs and eligibility that directly contradicts your testimony. The PBO's findings identify rejected claimants as among those receiving IFHP benefits and project associated costs accordingly. The PBO does not have a partisan interest. The PBO has an institutional interest in accuracy.

Source Two — Health Canada's own program documentation: Health Canada's published documentation describing IFHP coverage tiers explicitly outlines the categories of individuals eligible for benefits. Rejected asylum claimants are included. This documentation is publicly accessible on the Government of Canada's website. It is your government's own published material.

Source Three — IRCC's own program guidelines: Your department — the department over which you have direct ministerial authority — publishes guidelines that explicitly list rejected asylum claimants as eligible for the Expanded IFHP tier during their stay pending removal. This includes dental coverage, vision care, prescription drug coverage, and mental health services.

Minister Diab, these are benefits that millions of Canadian citizens and permanent residents cannot access through their provincial health plans. Many Canadians pay out-of-pocket for dental care. Many Canadians forgo prescription medications because they cannot afford them. Many Canadians are on months-long wait lists to access mental health services.

We are not asserting that rejected asylum claimants should or should not receive these benefits — that is a policy question on which Canadians reasonably disagree, and it is a question Parliament should openly debate. What we are asserting, and what the evidence establishes without equivocation, is that you told Parliament they were not receiving those benefits when they were — and when your own department's guidelines said they were.

That is not a policy disagreement. That is a false statement before a parliamentary committee.

3. Compelled Return to Committee: A Rare and Serious Parliamentary Rebuke

The consequences of your October 29, 2024 testimony did not end with the hearing itself. The committee found your testimony so materially incomplete that Conservatives successfully moved to compel your return. You were recalled to committee.

This is not a routine procedural occurrence. Ministers are not routinely recalled to committees because their testimony was found wanting. This was an extraordinary parliamentary rebuke — a formal finding, through democratic process, that what you told Parliament was insufficient to meet Parliament's legitimate need for accurate and complete information.

We acknowledge that you returned. That matters. Refusing to return would have constituted a complete breakdown of the ministerial accountability relationship, and your compliance with the recall prevented that outcome. This is noted, and it is given appropriate weight in how we have framed this letter rather than escalating to a resignation demand.

But returning to a committee to correct inadequate testimony is the floor of ministerial accountability — not the ceiling. Returning to committee is what is expected. It does not erase the fact that the inadequate testimony was given. It does not erase the impact on Parliament's ability to scrutinise your file. And it does not answer the central question: how did the Minister responsible for IRCC come to tell a parliamentary committee something that IRCC's own guidelines contradict?

4. No Healthcare Capacity Modelling

There is no publicly available evidence — none — that your department has commissioned or released any modelling of how immigration levels interact with provincial healthcare system capacity.

This is a planning failure of significant consequence. Canada's healthcare system is provincial in its delivery but federal in many of its structural inputs, including population growth driven by immigration. The question of how many people the healthcare system can absorb — in terms of physician availability, hospital capacity, specialist wait times, and community health infrastructure — is not a boutique policy concern. It is a foundational question that should sit at the centre of any responsible immigration levels planning exercise.

Instead, Canada has set immigration targets without this analysis. The downstream consequences are being borne by Canadians who cannot find a family doctor, who wait years for specialists, and who are told there is no capacity in the system for them — while the federal government continues to add to that system's patient population without modelling the impact of doing so.

The laughing dismissal of this concern in committee was troubling. The absence of any modelling to inform the answer is a policy failure with real human costs.

5. Removal Backlogs and Extended IFHP Benefit Receipt

Canada's removal backlog — the accumulation of rejected asylum claimants who have exhausted their appeals and been ordered removed but who remain in Canada pending enforcement action — continues to grow. During the period that rejected claimants remain in Canada pending removal, they remain eligible for IFHP benefits under the Expanded tier, including the dental, vision, prescription drug, and mental health services your testimony suggested they could not access.

The practical consequence is that the removal backlog is not merely an enforcement failure. It is a cost driver. Every day the backlog grows is a day that IFHP benefit expenditure on rejected claimants continues. The Parliamentary Budget Officer's analysis makes the fiscal trajectory of this program visible. What it cannot make visible — because no such modelling has been released — is any ministerial plan to address it.

A minister who cannot accurately describe who receives IFHP benefits, and who has no credible public plan for reducing the backlog that extends those benefits, is not in command of her file.

6. Judicial Sentencing Patterns and the IRPA Deportation Threshold Gap

The evidence before us also includes concern about documented judicial sentencing patterns in which sentences for non-citizen offenders are calibrated to fall below the thresholds in the Immigration and Refugee Protection Act (IRPA) that would trigger mandatory deportation proceedings. To be direct: this appears to be a pattern in which sentences are structured specifically to avoid the immigration consequences that Parliament intended to attach to serious criminality.

We are not calling for interference with judicial independence. Judges are independent, and that independence is fundamental to the rule of law in Canada. But ministerial independence from judicial decisions does not mean ministerial silence in the face of evidence that Parliament's statutory intent is being systematically circumvented. You are the minister responsible for IRPA. When evidence emerges that the deportation thresholds Parliament embedded in that statute are being avoided through sentencing patterns, you have an obligation to examine that evidence and to consider whether legislative or policy action is warranted.

To our knowledge, you have not publicly acknowledged this concern, nor taken any documented steps to address it. That silence is itself a failure of ministerial responsibility.

II. What We Are Asking: Six Specific Demands

Minister Diab, we are not asking for your resignation — not yet, and not in this letter. We are asking for something more demanding than resignation: accountability, accuracy, and action. These are the things resignation does not require.

We ask the following of you, by the deadlines specified:

Demand 1 — Correct the Record Before Parliament

Deadline: 30 days (March 30, 2026)

Appear before the Standing Committee on Health, or table a written statement in Parliament, that explicitly acknowledges and corrects your October 29, 2024 testimony regarding IFHP eligibility for rejected asylum claimants. The correction must be specific. It must identify the precise claim you made that was inaccurate. It must cite the IRCC program guidelines, Health Canada documentation, and PBO analysis that establish the accurate position. And it must explain — with specificity — how you came to give the committee information inconsistent with your own department's published guidelines.

Canadians do not require a political performance of contrition. They require an accurate account of who receives IFHP benefits, at what cost, under what authority, and with what justification.

Demand 2 — Commission and Release Healthcare Capacity Modelling

Deadline: 90 days (May 29, 2026)

Commission and commit to a public release date for a comprehensive healthcare capacity impact assessment examining how current and projected immigration levels interact with provincial healthcare system capacity. This assessment must include, at minimum:

- Physician-to-population ratio projections under current immigration levels
- Emergency department capacity modelling by region
- Specialist wait time projections
- Community health infrastructure requirements
- A methodology section that can be independently reviewed

This modelling should have been done before immigration targets were set. It has not been done. The responsible course now is to do it and to publish it — not to continue setting targets in the absence of this analysis.

Demand 3 — Release a Full IFHP Cost and Eligibility Accounting

Deadline: 60 days (April 29, 2026)

Release a comprehensive public accounting of IFHP benefit expenditures for the past five fiscal years, broken down by recipient category — including rejected asylum claimants — and by benefit type. This accounting should include the average duration of benefit receipt for rejected claimants and the projected cost trajectory under current removal backlog conditions.

If the policy of extending IFHP benefits to rejected asylum claimants is justified, justify it publicly. If it is not justified, reform it. What is not acceptable is continuing to administer a program of this fiscal and social significance in the absence of transparent public accounting, particularly when the minister responsible has told Parliament something that contradicts her own department's guidelines.

Demand 4 — Table a Credible Removal Backlog Reduction Plan Before Parliament

Deadline: 90 days (May 29, 2026)

Develop and table before Parliament a specific, funded, time-bound plan for reducing the removal backlog for rejected asylum claimants. The plan must include:

- Current backlog figures and growth rate
- Specific numerical targets for backlog reduction
- The resources required and how they will be funded
- A timeline with measurable milestones
- A reporting mechanism so Parliament can track progress

A minister who cannot tell Parliament how many rejected claimants are awaiting removal, for how long they have been waiting, and what the plan is to enforce Parliament's decisions through the refugee determination system, is not meeting the basic operational obligations of her portfolio.

Demand 5 — Acknowledge and Respond to the IRPA Sentencing Pattern Concern

Deadline: 60 days (April 29, 2026)

Provide a public ministerial statement acknowledging the concern regarding judicial sentencing patterns that calibrate below IRPA deportation thresholds for non-citizen offenders. The statement should:

- Acknowledge that this concern has been raised and warrants examination
- Describe what analysis, if any, IRCC has conducted on this question
- State whether the government intends to examine whether IRPA amendments are warranted to ensure Parliament's statutory intent is achieved

We are not asking you to compromise judicial independence. We are asking you to do your job as the minister responsible for the statute that Parliament enacted to govern these consequences.

Demand 6 — Appear Publicly to Answer Questions on These Matters

Deadline: 30 days (March 30, 2026)

Commit to and schedule a public forum — a parliamentary committee appearance, a press conference, or a publicly accessible town hall — specifically dedicated to the questions raised in this letter. Answer them in public, on the record, where Canadians can hear your responses.

Ministerial accountability is not discharged through press releases or carefully managed messaging. It is discharged by answering hard questions in public with accuracy and specificity. The committee appearance of October 29, 2024 established that you are capable of appearing before Parliament. We are asking you to return — fully prepared, fully accurate, and fully accountable.

III. What Happens Next

Minister Diab, we want to be clear about what stopbleeding.ca will do with your response — or the absence of one.

If you respond to these demands with specificity, accuracy, and in good faith, we will publish your response in full, without editorial interference, and we will update our evaluation of your performance accordingly. We hold ourselves to the same standard we hold you: if the evidence changes, our conclusions change.

If you do not respond within the deadlines specified above, or if your response is evasive, incomplete, or fails to address the substance of the concerns raised here, we will escalate. A formal resignation demand, accompanied by the full evidentiary record of your conduct as Minister, will be prepared and published. That document will be distributed to media, to your constituents in Halifax West, and to every parliamentary committee with oversight of your portfolio. It will be on the public record.

We do not make this statement as a threat. We make it as a statement of fact about how accountable government works: failure to respond to legitimate accountability demands is itself a data point that Canadians and Parliament are entitled to consider.

IV. A Final Word

Minister Diab, Canada's immigration system is one of the most complex and consequential policy responsibilities in this government. It touches on who we are as a country, what we owe to people fleeing persecution, and what obligations we hold to the Canadians who are already here — including the six million who cannot find a family doctor, and the many more who cannot afford dental care, prescription drugs, or mental health services.

These are not simple tensions. They do not admit of simple answers. And we acknowledge that many of the structural problems you inherited were not created by

you.

But the false testimony before the Health Committee was not inherited. The laughing dismissal of a question about healthcare capacity was not inherited. The absence of healthcare capacity modelling was a choice made during your tenure. These are your responsibilities.

Canada deserves a minister who will engage honestly with the hardest questions in her portfolio — not deflect them with laughter, not misrepresent program rules to Parliament, and not leave the fiscal and policy consequences of her decisions unmodelled and unaccounted for.

We are asking you to be that minister. The opportunity remains open.

The clock is running.

On behalf of concerned Canadians,

The Chief Executive

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This letter is published in full on stopbleeding.ca and is available to all Canadians, all media outlets, and Minister Diab's office. stopbleeding.ca is a non-partisan citizen accountability platform. This letter is based entirely on publicly verifiable evidence cited in full below. We invite scrutiny of our evidence and will publish any factual corrections submitted by Minister Diab's office or any reader who identifies a verifiable error.

Basis for This Letter

Field	Detail
Evaluation ID	EVAL-2026-001
Overall Rating	D — Poor (3.9/10)
Lowest Score	Transparency & Ethics: 2/10
Primary Trigger	False parliamentary testimony contradicted by PBO, Health Canada, and IRCC documentation

Key Failures Cited

1. False or materially misleading testimony before the Standing Committee on Health on October 29, 2024, stating that rejected asylum claimants are not eligible for premium IFHP benefits — contradicted by PBO, Health Canada, and IRCC
2. Laughing dismissal of healthcare capacity question while six million Canadians have no family doctor
3. No healthcare capacity modelling conducted or released to inform immigration levels planning
4. Compelled return to committee — an extraordinary and rare parliamentary rebuke
5. Growing removal backlog for rejected asylum claimants continuing to receive premium IFHP benefits
6. No documented ministerial response to judicial sentencing patterns circumventing IRPA deportation thresholds
7. Transparency and Ethics score of 2/10 — reflecting the gravity of providing Parliament with false information

Distribution

- Minister Diab's constituency office — Halifax West
- Minister Diab's ministerial office — Immigration, Refugees and Citizenship Canada
- stopbleeding.ca — full publication
- House of Commons Standing Committee on Health
- Parliamentary Budget Officer's office
- National media distribution
- Nova Scotia regional media distribution

Demand Deadlines Summary

Demand	Deadline	Date
1. Correct the Record	30 days	March 30, 2026
2. Healthcare Capacity Modelling	90 days	May 29, 2026

Demand	Deadline	Date
3. IFHP Cost Accounting	60 days	April 29, 2026
4. Removal Backlog Plan	90 days	May 29, 2026
5. IRPA Sentencing Response	60 days	April 29, 2026
6. Public Forum Appearance	30 days	March 30, 2026

Hard deadline for initial response: March 30, 2026

Escalation if no response: Formal resignation demand

Evidence Cited

1. House of Commons Standing Committee on Health — Testimony Transcript, October 29, 2024
2. Parliamentary Budget Officer — Interim Federal Health Program Cost Analysis
3. Health Canada — Interim Federal Health Program Documentation, Coverage Tiers
4. IRCC Program Guidelines — IFHP Expanded Tier Eligibility
5. Health Committee Recall Proceedings — Conservative Motion to Compel Minister's Return
6. IRCC Removal Backlog Statistics — 2024-2025
7. Canadian Medical Association — Physician Shortage and Family Doctor Access Data
8. stopbleeding.ca — Investigation INV-2025-007: *Laughing While Canadians Wait: Minister Diab's Healthcare Evasion and the IFHP Cover-Up*
9. stopbleeding.ca — Editorial Analysis: *Lena Diab Under the Microscope: A Comprehensive Assessment*
10. stopbleeding.ca — MP Evaluation Report EVAL-2026-001: Lena Diab